



MeticleCare

NHS Trust Pilot Proposal — Discharge-to-Assess Pathways

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Prepared for: [NHS Trust name — to be completed]

Applicant: MeticleCare Ltd

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SECTION 1

Executive Summary

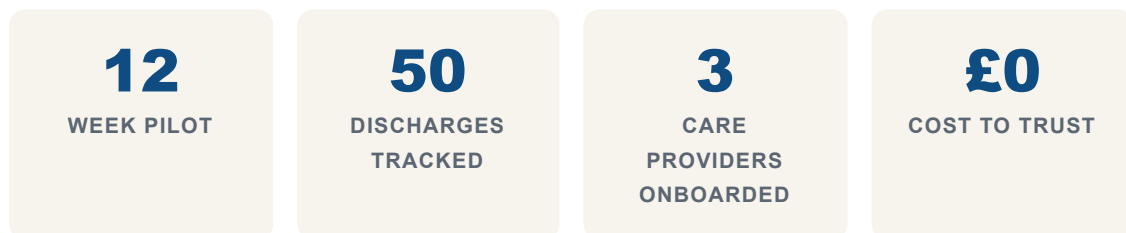
The proposal in one page

The Problem

Delayed discharges cost the NHS approximately **£400–£500 per patient per day**. In England, there are typically **12,000–14,000 delayed discharge days per week**, representing an annual cost of **£2.5–£3.5 billion**. The primary cause is the gap between hospital readiness to discharge and community readiness to receive — patients are clinically optimised but lack arranged care and support.

The Proposal

MeticleCare proposes a **12-week pilot partnership** with [Trust name] to demonstrate how a connected care management platform can reduce delayed discharges by improving information flow between hospital discharge teams and community care providers.



What MeticleCare Offers

- **Connected information flow:** Hospital discharge teams share patient summary, care needs, and medication information directly with receiving care providers through one platform
- **Care plan visibility:** Community care providers can access discharge summaries, care plans, and medication records before the patient arrives
- **Real-time status tracking:** Trust discharge coordinators can see when care is arranged, when the patient is received, and when assessment is completed
- **Family portal:** Families can access care notes and updates, reducing phone calls to wards and improving satisfaction
- **Compliance evidence:** D2A pathway compliance is automatically documented for ICB reporting

Expected Outcomes

METRIC	CURRENT	TARGET (PILOT)
Delayed discharge days (per 50 discharges)	~150 days	<75 days (50% reduction)
Time to care arrangement	48–72 hours	<24 hours
Information completeness at handover	60%	>90%
Family satisfaction (NPS)	Unknown	>40
Cost to trust	—	£0 (pilot period)

Key proposition: MeticleCare provides the digital infrastructure to connect hospital discharge teams with community care providers — the missing link in the D2A pathway. This pilot demonstrates whether connected information flow reduces delayed discharges and improves patient outcomes.

SECTION 2

The Discharge-to-Assess Challenge

Understanding the current D2A pathway and its failures

2.1 What is Discharge-to-Assess?

The Discharge-to-Assess (D2A) model allows patients who are clinically optimised to be discharged from hospital before their long-term care needs are formally assessed. Assessment happens in the community — in the patient's own home or in a care setting. The model has three pathways:

Pathway 1: Home with support

Patient returns home with short-term care and support (reablement, rehabilitation). Assessment of long-term needs happens at home. This is the most common pathway — approximately 70% of D2A discharges.

Pathway 2: Short-term care placement

Patient is placed in a care home or residential setting for short-term care (typically up to 6 weeks). Long-term care needs are assessed during the placement. Funded by the local authority or NHS.

Pathway 3: Intermediate care / reablement

Patient receives NHS-funded intermediate care or reablement (typically up to 6 weeks). Focus on rehabilitation and recovery. Assessment happens during or after the placement.

2.2 Where the Pathway Fails

Despite the D2A model being in place since 2020, delayed discharges remain a persistent problem. The failures are not clinical — they are informational and logistical:

FAILURE POINT	IMPACT	METICLECARE SOLUTION
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Information gap	Receiving care provider doesn't have patient information until arrival	Pre-arrival care plan and medication sharing
Coordination delay	Care provider takes 48–72 hours to arrange care after referral	Real-time availability tracking and instant notification
Medication mismatch	Medication records not transferred accurately, causing delays	Digital MAR transfer with verification
Family confusion	Families not informed of discharge plan, causing delays	Family portal with real-time updates
Assessment lag	Long-term needs assessment delayed by weeks	Structured assessment data captured at point of care
Reporting gap	ICB cannot track D2A pathway compliance in real time	Automated D2A compliance dashboard

2.3 The Cost of Failure



"The D2A model is sound in principle. The failures are not clinical — they are informational. Care providers don't have patient information until arrival. Families aren't informed until the last minute. Assessment data isn't captured until weeks later. The digital infrastructure to support the pathway doesn't exist."

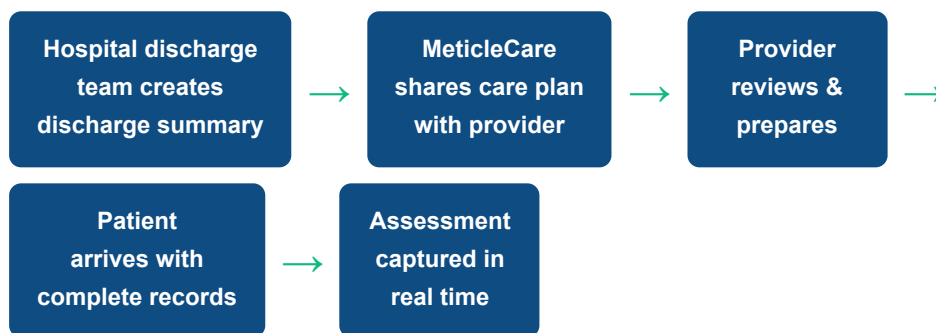
SECTION 3

How MeticleCare Supports D2A Pathways

Platform capabilities mapped to discharge pathway requirements

3.1 Connected Information Flow

MeticleCare provides the digital infrastructure to share information between hospital discharge teams and community care providers:



3.2 Platform Features for D2A

D2A REQUIREMENT	METICLECARE FEATURE	BENEFIT
Patient summary transfer	Discharge summary module with structured data	Receiving provider has full context before arrival
Care plan sharing	Care plans accessible to authorised providers	Care is arranged based on actual needs, not assumptions
Medication transfer	eMAR with discharge medication list	Medication records accurate from day one
Family communication	Family portal with care notes and updates	Families informed, reducing anxiety and phone calls
Care provider availability	Multi-provider capacity tracking	Discharge coordinator sees available beds in real time
Assessment capture	Structured assessment forms (capacity, wellbeing, health)	Assessment data captured at point of care, not weeks later

Incident reporting	Digital incidents with AI triage	Safeguarding concerns flagged immediately
Compliance tracking	D2A pathway compliance dashboard	ICB can track compliance in real time
Real-time status	Live status tracking for discharge coordinators	Trust knows exactly where every patient is in the pathway
Data export	CSV/JSON export for ICB reporting	Automated reporting for system-level oversight

3.3 User Roles in the D2A Pilot

ROLE	ORGANISATION	METICLECARE ACCESS
Discharge coordinator	NHS Trust	Create discharge summaries, track status, view provider availability
Ward nurse / therapist	NHS Trust	Complete discharge assessment, share care plan, record medications
Care manager	Community care provider	Receive discharge summary, prepare care plan, allocate staff
Care worker	Community care provider	Receive patient, record daily notes, complete assessments
Family member	Patient's family	View care notes, updates, and assessment progress (consent-gated)
ICB analyst	Integrated Care Board	View D2A compliance dashboard, export data for reporting

3.4 Data Sharing Agreement

MeticleCare operates under a Data Processing Agreement compliant with UK GDPR. Key provisions for the pilot:

- **Data controller:** NHS Trust (for patient data) and MeticleCare Ltd (for platform data)
- **Data processor:** MeticleCare Ltd processes patient data only on the Trust's instructions
- **Data residency:** All data hosted in UK data centres (AWS London / Azure UK South)

- **Encryption:** TLS 1.3 in transit, AES-256-GCM at rest
- **Access control:** Role-based access, per-request permission checks, audit logging
- **Data retention:** Patient data retained for pilot duration + 30 days, then deleted or exported
- **Breach notification:** 72-hour notification to Trust and ICO

SECTION 4

Pilot Design

12-week structured pilot with clear success criteria

4.1 Pilot Objectives

1. Demonstrate that connected information flow reduces time to care arrangement
2. Measure the impact on delayed discharge days
3. Assess information completeness at patient handover
4. Evaluate family satisfaction with the discharge process
5. Test D2A pathway compliance reporting for ICB oversight
6. Identify integration requirements for NHS system connectivity

4.2 Pilot Scope

PARAMETER	DETAIL
Duration	12 weeks
Patient cohort	50 discharges (Pathways 1, 2, and 3)
Ward(s)	[To be agreed — recommend geriatric assessment unit or stroke rehabilitation]
Care providers	3 community care providers (mix of residential and home care)
Family members	Up to 50 families (consent-gated access)
ICB involvement	Observer access to compliance dashboard

4.3 Pilot Phases

Weeks 1–2: Setup & Onboarding

Configure MeticleCare for the trust. Onboard discharge team (5–10 staff). Onboard 3 care providers (2–3 staff each). Set up data sharing agreements. Configure D2A compliance dashboard.

Weeks 3–4: Training & Soft Launch

Training sessions for all users. First 5 discharges tracked (with parallel paper process). Identify and resolve integration issues. Baseline data collection.

Weeks 5–10: Active Pilot

All discharges tracked through MeticleCare. Weekly check-ins with pilot coordinator. Data collection on all metrics. Issue resolution and process refinement.

Weeks 11–12: Evaluation & Reporting

Final data collection. Pilot evaluation report. Presentation to trust leadership. Decision: full deployment, extended pilot, or decline.

4.4 Success Criteria

METRIC	BASELINE (PRE-PILOT)	TARGET (PILOT)	MEASUREMENT
Delayed discharge days (per 50 discharges)	~150 days	<75 days	Trust discharge data
Time to care arrangement	48–72 hours	<24 hours	System timestamps
Information completeness at handover	60%	>90%	Structured checklist
Medication accuracy at transfer	85%	>98%	eMAR verification
Family satisfaction (NPS)	Unknown	>40	End-of-pilot survey
Staff satisfaction (NPS)	Unknown	>30	End-of-pilot survey
D2A pathway compliance	70%	>90%	Compliance dashboard
Cost to trust	—	£0	No charge during pilot

SECTION 5

Implementation Plan

Detailed activities and responsibilities

5.1 Week-by-Week Plan

WEEK	ACTIVITY	METICLECARE	TRUST	CARE PROVIDERS
1	Platform configuration	Configure D2A workflow, user roles, permissions	Provide ward details, staff list	—
1	Data sharing agreement	Finalise DPA	Legal review, sign	Sign DPA
2	Provider onboarding	Onboard 3 care providers	Introduce providers	Complete onboarding
2	Discharge team training	Deliver training (2 sessions)	Arrange attendance	—
3	Provider training	Deliver training (2 sessions)	—	Attend training
3	Soft launch (5 discharges)	Support, monitor, resolve issues	Process 5 discharges via platform	Receive 5 patients via platform
4	Soft launch review	Analyze issues, refine process	Feedback session	Feedback session
5–10	Active pilot	Weekly check-ins, issue resolution	Process all discharges via platform	Receive all patients via platform
8	Mid-pilot review	Present interim findings	Review progress	Review progress
11	Data collection close	Final data extraction	Confirm completion	Confirm completion

12	Evaluation report	Deliver pilot evaluation	Review, present to leadership	Review findings
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5.2 Resources Required

MeticleCare Will Provide

- Dedicated pilot coordinator
- Platform access for all users
- Onboarding and training
- Weekly check-ins and support
- Data analysis and reporting
- Pilot evaluation report
- All at no cost during pilot

Trust Will Provide

- Discharge coordinator access (2–3 users)
- Ward staff access (5–10 users)
- Patient consent for data sharing
- Baseline discharge data
- Feedback sessions (30 min/week)
- Leadership sponsor for pilot
- ICB observer access

5.3 Risk Register

RISK	LIKELIHOOD	IMPACT	MITIGATION
Staff resist using new system	Medium	High	Dedicated training, parallel running in weeks 3–4, champion identification
Patient consent issues	Medium	Medium	Clear consent process, opt-out mechanism, GDPR compliance
Integration with NHS systems	High	Medium	Manual data entry initially, NHS integration planned for post-pilot
Low patient volume	Low	Medium	Flexible pilot duration, extend if needed
Data breach	Low	High	UK hosting, encryption, access controls, audit logging, breach notification

SECTION 6

Commercial Model

Pilot terms and post-pilot pricing

6.1 Pilot Terms

TERM	DETAIL
Duration	12 weeks
Cost to trust	£0
Cost to care providers	£0
Platform access	Full Professional tier
Support	Dedicated coordinator + weekly check-ins
Data	Patient data returned/deleted at pilot end
Commitment	None — walk away at any time

6.2 Post-Pilot Pricing (if deployed)

COMPONENT	MONTHLY COST	NOTES
Trust licence (discharge coordination)	£499/month	Unlimited discharge coordinators
Care provider licence (per provider)	£299/month	Professional tier, unlimited staff
Family portal (per patient)	£0	Included in care provider licence
ICB dashboard	£0	Included in trust licence

Value proposition: If the pilot achieves a 50% reduction in delayed discharge days (from 150 to 75 days for 50 patients), the saving is approximately **£37,500–£46,875** (75 days × £500/day). The annual post-pilot cost for trust + 3 providers is approximately **£14,364**. ROI: **2.6–3.3x** in the first year.

6.3 NHS Procurement Pathway

- **Pilot:** No procurement required (free trial, no contract)
- **Short-term deployment:** NHS Light Touch Process (under £138K threshold)
- **Long-term contract:** Competitive dialogue or restricted procedure (over £138K)
- **Framework:** NHS Digital / Technology Framework (if available)

SECTION 7

Evidence & Evaluation Framework

How success will be measured and reported

7.1 Data Collection

DATA POINT	SOURCE	FREQUENCY
Discharge date/time	Trust discharge system	Per patient
Care arrangement date/time	MeticleCare timestamps	Per patient
Patient arrival at provider	MeticleCare timestamps	Per patient
Assessment completion date	MeticleCare timestamps	Per patient
Information completeness	Structured checklist in MeticleCare	Per patient
Medication accuracy	eMAR verification	Per patient
Family portal usage	MeticleCare analytics	Weekly
Staff satisfaction	Survey (Week 6 and Week 12)	Twice
Family satisfaction	Survey (Week 12)	Once

7.2 Evaluation Report

At Week 12, MeticleCare will deliver a pilot evaluation report including:

- Quantitative analysis of all metrics against success criteria
- Qualitative feedback from discharge coordinators, care staff, and families
- Case studies of successful discharges
- Identification of process improvements and integration requirements
- Recommendations for full deployment
- Cost-benefit analysis for trust procurement

7.3 NHS Evidence Standards

STANDARD	METICLECARE APPROACH
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Clinical safety (DCB0129/DCB0160)	Clinical safety case report to be developed during pilot
Data security (DSPT)	DSPT toolkit completed; annual assessment planned
Interoperability (FHIR)	FHIR API planned for NHS Spine integration (post-pilot)
Accessibility (WCAG 2.1)	AA compliance targeted; keyboard navigation, contrast ratios
Usability (ISO 62366)	User-centred design; pilot feedback informs improvements

SECTION 8

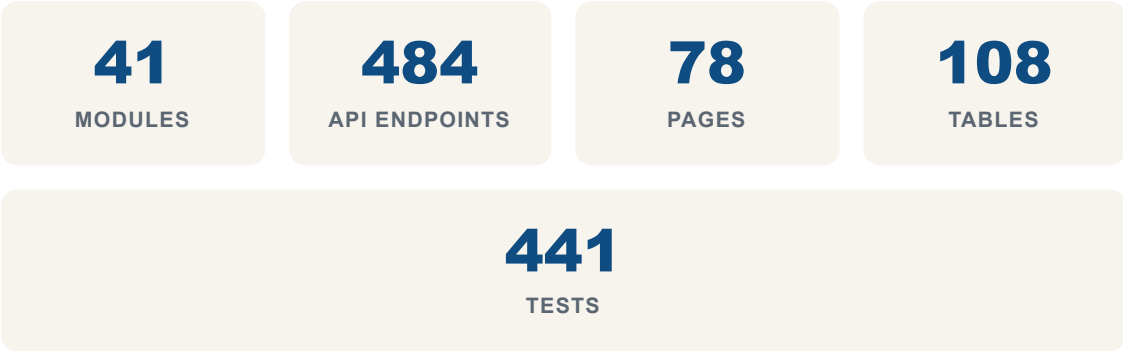
About MeticleCare

Company and platform credentials

8.1 Company Overview

MeticleCare Ltd is a UK-registered company developing an AI-first care management platform for UK supported-living and domiciliary care providers. The platform is deployed in production with 41 backend modules, 484 API endpoints, 78 frontend pages, and 108 database tables.

8.2 Platform Capabilities



8.3 Security & Compliance

Requirement	Status
UK GDPR compliance	Implemented
Data Processing Agreement	Template ready
UK data residency	AWS London / Azure UK South
Encryption (transit)	TLS 1.3
Encryption (rest)	AES-256-GCM
Authentication	JWT + MFA (TOTP)
Access control	RBAC + per-request permissions
Audit logging	40+ mutation points logged

Breach notification	72-hour process
Cyber Essentials	Target Q4 2026
DSPT	Toolkit completed

8.4 Regulatory Framework Support

REGULATOR	JURISDICTION	STATUS
CQC	England	Implemented
CIW	Wales	Supported
Care Inspectorate	Scotland	Supported
RQIA	Northern Ireland	Supported

8.5 Contact

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Platform demo available on request.